

Our Ref: (T) MN/CD/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CF88, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS], Totton, Southampton, [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Total colectomy and pouch formation

Short bowel syndrome

DVTs on warfarin

Recurrent pouchitis

I reviewed this lady in the Nutrition Clinic. Her weight is currently 116 kg and has been stable while. She has had episodes of water and salt depletion in the past but is maintaining her hydration currently. Her urinary sodium in September was 72 and the rest of her bloods were also normal. With regards to the pouchitis she usually takes ciprofloxacin 500 mg twice a day but has had metronidazole over the last month.

She is keen to try VSL#3 which was recommended by [PROVIDER-NAME] in the last clinic and I have a recommendation for prescription for this to try for one month. This may reduce her antibiotic use. I have advised her to stop the metronidazole now as she would be at risk of peripheral neuropathy with long-term metronidazole use and she will switch back to Cipro and try the VSL3.

I have discussed her case with [PROVIDER-NAME] and we will review her again in six months' time with further bloods and urinary sodium.

Yours sincerely

[PROVIDER-NAME]

Specialist Registrar in Gastroenterology.

Copy to:

Miss [NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]